

Wisconsin Department of Health Services

STATEMENT OF DEFICIENCIES AND PLAN OF CORRECTION		(X1) PROVIDER/SUPPLIER/CLIA IDENTIFICATION NUMBER: 0017158	(X2) MULTIPLE CONSTRUCTION A. BUILDING: _____ B. WING: _____	(X3) DATE SURVEY COMPLETED R-C 07/16/2026
NAME OF PROVIDER OR SUPPLIER ALICE & LOUISES II		STREET ADDRESS, CITY, STATE, ZIP CODE 8679 STATE HWY 186 ARPIN, WI 54410		
(X4) ID PREFIX TAG	SUMMARY STATEMENT OF DEFICIENCIES (EACH DEFICIENCY MUST BE PRECEDED BY FULL REGULATORY OR LSC IDENTIFYING INFORMATION)	ID PREFIX TAG	PROVIDER'S PLAN OF CORRECTION (EACH CORRECTIVE ACTION SHOULD BE CROSS-REFERENCED TO THE APPROPRIATE DEFICIENCY)	(X5) COMPLETE DATE
{N 000}	Initial Comments On 07/07/2026 and 07/08/2026, Surveyors conducted onsite visits at Alice and Louises II to investigate 1 complaint, conduct a standard survey and complete a verification visit. Additional information was collected through 07/16/2026. One (1) of 1 complaint was substantiated. Nine (8) of 14 previous deficiencies were identified as repeat violations. A total of 15 deficiencies were identified. Under statutory provisions of WI Stat. Ch. 50 a \$200 revisit fee is being assessed. Census: 6	{N 000}		
N 196	83.14(2)(a) Licensee ensures facility complies with laws The licensee shall ensure the CBRF and its operation comply with all laws governing the CBRF. This Rule is not met as evidenced by: Based on record review, observation and interview, the licensee did not ensure compliance with special orders or with correction of 9 of 14 deficiencies from the previous survey. See Statement of Deficiency (SOD) UK3M14, dated 08/05/2026. Findings include: Prior to conducting the onsite visit, Surveyor reviewed the provider's compliance history. The provider was licensed on 02/26/2020. Licensee A is also designated as the	N 196		

LABORATORY DIRECTOR'S OR PROVIDER/SUPPLIER REPRESENTATIVE'S SIGNATURE

TITLE

(X6) DATE

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N 196	Continued From page 1 administrator. --SOD UK3M11, dated 09/11/2023 resulted in 1 of 1 complaints substantiated and 5 deficiencies. --SOD UK3M12, dated 07/02/2024 resulted in 2 of 2 complaints substantiated and 18 deficiencies. --SOD UKEM13, dated 07/25/2025 resulted in 2 of 2 complaints substantiated and 14 deficiencies. This SOD was issued to the provider on 08/05/2025 with special orders that read in part, "Pursuant to Wis. Stat. § 50.03(5g)(b)6., the licensee shall develop and implement corrective measures to ensure each resident is treated with courtesy, respect, and allowed to make decisions related to care, activities, and daily routines which enhance the resident's self reliance and support the resident's autonomy and decision making. In the absence of a documented treatment plan, based on clinical or therapeutic indications, no restriction on a resident's freedom of choice (e.g., visitors, restricted access to food or beverages) will be arbitrarily imposed. WITHIN 45 DAYS, the licensee will obtain resident rights training for all current resident care staff. Resident rights training will be provided by a qualified professional (ombudsman, clinical social worker, registered nurse, health care administrator)..- The trainer will be provided a copy of Statement of Deficiency (SOD) UK3M13, and this notice and order letter before providing services. - Training will be documented in personnel files and will include the date/duration of training, the signature/qualifications of the instructor, and an outline of course content." On 07/07/2026 and 07/08/2026, Surveyors	N 196		

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N 196	Continued From page 2 conducted onsite visits to the facility to investigate 1 complaint, conduct a standard survey and to verify correction of the 14 deficiencies identified in SOD UKEM13. As a result of the survey; the complaint was substantiated, 8 of 14 previous deficiencies were identified as repeat violations and a total of 15 deficiencies were identified. The uncorrected deficiencies included Rights of Residents: Self-Determination and impacted Resident 7 whose access to snacks was restricted. On 07/07/2026, Surveyors interviewed Manager S and Co-Owner K. Surveyor reviewed the special orders and the requirement for resident rights training documented in personnel records to include the date, duration, signature of the instructor and an outline of the course content. Co-Owner S recalled Ombudsman U provided training in September 2025 but s/he did not recall the exact date or have any documentation related to the training. Between 07/07/2026 and 07/14/2026, Surveyor had email correspondence with Ombudsman U. Ombudsman U wrote that s/he provided resident rights training at the facility on 08/27/2025 but was not given a copy of SOD UKEM13 and "had no idea the training was in response to an order." Ombudsman U indicated s/he did not have the names of the staff who attended the trainings and wrote, "I expect the facility to document who received training on given date.." On 07/15/2026, Surveyors interviewed Licensee A and reviewed the aforementioned concerns. Licensee A said as a result of the survey s/he recognizes the need to be onsite more frequently to effectively oversee day to day operations.	N 196			

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N 196	Continued From page 3 Licensee A stated since the conclusion of the onsite visits the previous week, s/he had enrolled in the Department approved administrator course beginning in September 2026. Licensee A said s/he wanted to refresh his/her knowledge of DHS 83 requirements and address systems concerns. Cross Reference: N0220 DHS 83.17(2)(a) Employees Screened for Communicable Disease N0230 DHS 83.19 Orientation N0243 DHS 83.21(1)-(3) All Employee Training N0247 DHS 83.22 (1)-(4) Task Specific Training N0327 DHS 83.31(4)(b) Allowable Reasons For Involuntary Discharge N0328 DHS 83.31(4)(c) Involuntary Discharge Notice Requirements N0348 DHS 83.32(3)(d) Rights of Residents: Free From Mistreatment N0352 DHS 83.32(3)(h) Rights of Residents: Receive Medication N0355 DHS 83.32(3)(k) Rights of Residents: Self Determination N0381 DHS 83.35(1)(a) Pre-Admission And Ongoing Assessments N0385 DHS 83.35(2) Temporary Service Plan N0387 DHS 83.35(3)(b) Service Plan Development: Parties Involved N0389 DHS 83.35(3)(d) Service Plans Updated Annually Or On Changes N0397 DHS 83.36(1)(b) Qualified Staff in Charge, On Duty and Awake N0431 DHS 83.38(1)(g) Health Monitoring	N 196		
N 220	83.17(2)(a) Employees screened for communicable disease. The CBRF shall obtain documentation from a physician, physician assistant, clinical nurse	N 220		

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N 220	Continued From page 4 practitioner or a licensed registered nurse indicating all employees have been screened for clinically apparent communicable disease including tuberculosis. Screening for tuberculosis shall be conducted using centers for disease control and prevention standards. The screening and documentation shall be completed within 90 days before the start of employment. The CBRF shall keep screening documentation confidential, except the department shall have access to the screening documentation for verification purposes. This Rule is not met as evidenced by: Based on record review and interview, the provider did not ensure documentation was obtained from a physician, physician assistant, clinical nurse practitioner or licensed registered nurse indicating 3 of 3 employees reviewed (Caregiver R, Caregiver T or House Manager S) were screened for clinically apparent communicable disease, including tuberculosis (TB). Findings include: On 07/07/2026 at 8:50 AM, Surveyor requested employee files for Caregiver R, Caregiver T and House Manager S. House Manager S stated that documentation of his/her TB test and Caregiver T's TB test was not located in the employee files. House Manager S stated s/he would need to contact his/her medical provider to acquire his/her TB test documentation. On 07/07/2026 at 1:30 PM, Surveyor reviewed employee files for Caregiver R, Caregiver T and House Manager S for documentation to verify they were screened for clinically apparent	N 220		

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N 220	Continued From page 5 communicable disease, including TB. Caregiver R's hire date was 10/03/2025. His/her file did not contain documentation verifying s/he was screened for clinically apparent communicable disease, including TB. Caregiver T's hire date was 10/08/2025. His/her file did not contain documentation verifying s/he was screened for clinically apparent communicable disease, including TB. House Manager S's hire date was 08/20/2025. His/her file did not contain documentation verifying s/he was screened for clinically apparent communicable disease, including TB. ' On 07/07/2026 at 3:06 PM, Surveyor interviewed House Manager S and Co-Owner K. House Manager S verified that the employee files did not contain evidence that Caregiver R, Caregiver T and him/herself were screened for clinically apparent communicable disease including TB. House Manager S stated s/he could contact his/her medical provider to acquire documentation of his/her TB test, but s/he would not have time "until next week." Co-Owner K stated s/he would not be able to acquire Caregiver R's screenings as s/he was no longer employed and when asked if the employee files contained documentation of a communicable disease screening for Caregiver R, Caregiver T and House Manager S, Co-Owner K responded "No." Co-Owner K recalled an email that s/he received regarding communicable disease screening requirements and voluntarily showed Surveyor an email that s/he received on 02/25/2026 titled "Communicable Disease screening form." The email contained an attached form titled "F01679 Communicable Disease	N 220		

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N 220	Continued From page 6 Form" which read in part, " ...The Department requires that health care agencies or providers screen all health staff WITHIN 90 DAYS BEFORE DIRECT CONTACT AND PERIODICALLY, to ensure that staff is free of any communicable diseases before coming into contact with clients ..." Cross Reference: N0196 DHS 83.14(2)(a) Licensee Ensures Facility Complies with Laws	N 220		
N 230	83.19 Orientation Before an employee performs any job duties, the CBRF shall provide each employee with orientation training which shall include all of the following: (1) Job responsibilities; (2) Prevention and reporting of resident abuse, neglect and misappropriation of resident property; (3) Information regarding assessed needs and individual services for each resident for whom the employee is responsible; (4) Emergency and disaster plan and evacuation procedures under s. HFS 83.47(2); (5) CBRF policies and procedures; (6) Recognizing and responding to resident changes of condition. This Rule is not met as evidenced by: Based on record review and interview, the provider did not ensure 3 of 3 employees reviewed received all required orientation training prior to performing job duties. Caregiver R, Caregiver T and House Manager S did not have training on job responsibilities,	N 230		

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N 230	Continued From page 7 prevention and reporting of resident abuse, neglect and misappropriation, emergency and disaster plan and evacuation procedures, the needs and individual services for each resident for whom the employee was responsible prior to performing job duties, CBRF policies and procedures and recognizing and responding to resident changes of condition. Findings include: The facility is licensed to provide services for up to 8 residents who may be physically disabled, terminally ill, developmentally disabled, emotionally disturbed/mental illness, advanced aged and/or have Alzheimer's/dementia. On 07/07/2026, as part of the survey process, Surveyor requested training records from House Manager S and conducted a review of 3 employee records to verify compliance with all employee orientation training requirements. On 07/07/2026 at 11:23 AM, Surveyor reviewed the employee record for Caregiver R. The record for Caregiver R, hired 10/03/2025, contained a document titled "Staff Continuing Education Record" which read in part, " ... Train Start Date: 10/2025 Train End Date: 12/2025 ..." with documentation of training in medication administration, "DHS - Licensing Checklist", and personnel policy and procedures. The record did not contain evidence of orientation training on prevention and reporting of resident abuse, neglect and misappropriation, information regarding assessed needs and individual services for each resident, emergency and disaster plan and evacuation, CBRF policies and procedures and recognizing and responding to a change in condition.	N 230		

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N 230	Continued From page 8 On 07/07/2026 at 1:20PM, Surveyor reviewed the employee records for House Manager S. The record for House Manager S, hired 08/20/2025, contained a document titled "Staff Continuing Education Record" with documentation of training completed after 01/02/2026 in personnel policy and procedures, dementia care, fall prevention, hospice 101 and stroke management. The records did not contain evidence of orientation training on prevention and reporting of resident abuse, neglect and misappropriation, information regarding assessed needs and individual services for each resident, emergency and disaster plan and evacuation, CBRF policies and procedures and recognizing and responding to a change in condition. On 07/07/2026 at 1:30 PM, Surveyor reviewed the employee records for Caregiver T. The record for Caregiver T, hired 10/08/2025, contained a document titled "Staff Continuing Education Record" with documentation of training completed after 01/05/2026 in personnel policy and procedures, dementia care, fall prevention, hospice 101 and stroke management. The records did not contain evidence of orientation training on prevention and reporting of resident abuse, neglect and misappropriation, information regarding assessed needs and individual services for each resident, emergency and disaster plan and evacuation, CBRF policies and procedures and recognizing and responding to a change in condition. On 07/07/2026, at 1:45 PM, Surveyor interviewed House Manager S. S/he verified overseeing Caregiver T's training and that Caregiver T's first day working independently was 10/16/2025. S/he stated "we do not have base training" and that	N 230		

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N 230	Continued From page 9 new staff "work two or three shifts" for orientation training. S/he said that Caregiver T received training presented by an Ombudsman regarding abuse, neglect and misappropriation on 06/17/2026. When asked to provide documentation of the training including the date, duration, and staff present, s/he handed Surveyor an Ombudsman brochure. Surveyor went through the missing orientation topics with House Manager S. S/he stated staff review residents care plans during their first shift and that the emergency disaster protocol is posted by the medication cart for staff to review. When asked for documentation of orientation training, s/he stated there is "no documentation" regarding prevention and reporting of resident abuse, neglect and misappropriation, information regarding assessed needs and individual services for each resident, emergency and disaster plan and evacuation, CBRF policies and procedures and recognizing and responding to a change in condition. On 07/13/2026, Licensee A sent an email to Surveyors with a document titled "Training Checklist." The training checklist did not include evidence of a system to ensure all employees were trained in: job responsibilities; prevention and reporting of resident abuse, neglect and misappropriation of resident property; CBRF policies and procedures; or recognizing and responding to resident changes of condition. In addition, the checklist that was provided was blank and did not document orientation training for the 3 aforementioned caregivers. Cross Reference: N0196 DHS 83.14(2)(a) Licensee Ensures Facility Complies with Laws N0243 DHS 83.21(1)-(3) All Employee Training	N 230		

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N 230	Continued From page 10 N0247 DHS 83.22 (1)-(4) Task Specific Training	N 230		
N 243	83.21(1)-(3) All employee training. The CBRF shall provide, obtain or otherwise ensure adequate training for all employees in all of the following: Resident rights. Training shall include general rights of residents including rights as specified under s. DHS 83.32(3). Training shall be provided as applicable under ss. 50.09 and 51.61 and chs. 54, 55, and 304, Stats., and ch. DHS 94, depending on the legal status of the resident or service the resident is receiving. Specific training topics shall include house rules, coercion, retaliation, confidentiality, restraints, self-determination, and the CBRF 's complaint and grievance procedures. Residents ' rights training shall be completed within 90 days after starting employment. Client Group. Training shall be specific to the client group served and shall include the physical, social and mental health needs of the client group. Specific training topics shall include, as applicable: characteristics of the client group served, activities, safety risks, environmental considerations, disease processes, communication skills, nutritional needs, and vocational abilities. Client group specific training shall be completed within 90 days after starting employment. Client Group. In a CBRF serving more than one client group, employees shall receive training for each client group. Recognizing, preventing, managing, and responding to challenging behaviors. Specific training topics shall include, as applicable: elopement, aggressive behaviors, destruction of property, suicide prevention, self-injurious	N 243		

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N 243	Continued From page 11 behavior, resident supervision, and changes in condition. Challenging behaviors training shall be completed within 90 days after starting employment. This Rule is not met as evidenced by: Based on record review and interview, the provider did not ensure three of three employees reviewed obtained all training as required within 90 days after starting employment. Caregiver R, House Manager S and Caregiver T did not receive training in client group, resident rights or in recognizing preventing managing and responding to challenging behaviors within 90 days after starting employment. Findings include: The facility is licensed to provide services for up to 8 residents who may be physically disabled, terminally ill, developmentally disabled, emotionally disturbed/mental illness, advanced aged and/or have Alzheimer's/dementia. On 07/07/2026, as part of the survey process, Surveyor conducted a review of 3 employees to verify compliance with all employee training requirements. Surveyor requested training records from House Manager S for Caregiver R, House Manager S and Caregiver T. The records for Caregiver R (hired 10/03/2025), House Manager S (hired 08/20/2025) and Caregiver T (hired 10/08/2025) did not contain	N 243			

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N 243	Continued From page 12 evidence of training or evidence of meeting an exemption for client group, resident rights or challenging behaviors. On 07/07/2026 at 1:45 PM, Surveyor interviewed House Manager S. S/he stated, "it would not make sense" to train staff in challenging behaviors as they do not admit residents with challenging behaviors. S/he stated staff were trained in resident rights by an Ombudsman on 06/17/2026, but upon request was unable to provide documentation of which staff were present and the duration of the training. Surveyor noted the 06/17/2026 resident rights training exceeds the 90 day period for 3 out of 3 staff reviewed. House Manager S acknowledged that the employee files did not contain evidence that training was completed for all client groups served (physically disabled, terminally ill, developmentally disabled, emotionally disturbed/mental illness, advanced age and Alzheimer's/dementia). On 07/13/2026, Licensee A sent an email to Surveyors with a document titled, "Director Duties (ALL LOCATIONS) (MUST HAVE NEW COPIES OF ALL AT HOME OFFICE)...All staff must have completed Standard Precautions, Medication Administration, First Aid, Fire Safety, Resident Rights and Challenging Behaviors training within 90 days of hire, no exceptions. Also, must have Medication Administration certification and Standard Precautions before working any shift." The document that was provided was blank and did not include evidence the 3 aforementioned caregivers received all required training within 90 days. Cross Reference: N0196 DHS 83.14(2)(a) Licensee Ensures	N 243			

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N 243	Continued From page 13 Facility Complies with Laws N0230 DHS 83.19 Orientation N0247 DHS 83.22 (1)-(4) Task Specific Training	N 243		
N 247	83.22(1)-(4) Task specific training. The CBRF shall provide, obtain or otherwise ensure adequate training for employees performing job duties in all of the following: Assessment of residents. All employees responsible for resident assessment shall successfully complete training in the assessment of residents prior to assuming these job duties. Specific training topics shall include: assessment methodology, assessment of changes in condition, sources of assessment information, and documentation of the assessment. Individual service plan development. All employees responsible for service plan development shall successfully complete training in individual service plan development prior to assuming these job duties. Specific training topics shall include: identification of the resident ' s needs and desired outcomes, development of goals and interventions, service plan evaluation and review of progress. Provision of personal care. All employees responsible for providing assistance with activities of daily living shall successfully complete training prior to assuming these job duties. Specific training topics shall include, as appropriate: bathing, eating, dressing, oral hygiene, nail and foot care, toileting and incontinence care, positioning and body alignment, and mobility and transferring. Dietary training. All employees performing dietary duties shall complete dietary training within 90 days after assuming these job duties. Specific training topics shall include: determining	N 247		

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N 247	Continued From page 14 nutritional needs, menu planning, food preparation and food sanitation. This Rule is not met as evidenced by: Based on interview and record review the provider did not ensure 3 of 3 employees received all task specific training. Caregiver R, who also performed dietary duties, did not have training in activities of daily living or dietary services Caregiver T, who also performed dietary duties, did not have training in activities of daily living or dietary services. House Manager S, who also performed dietary duties, worked as a caregiver, and performed assessments, did not have training in activities of daily living, dietary services or assessment. Findings include: The facility is licensed to provide services for up to 8 residents who may be physically disabled, terminally ill, developmentally disabled, emotionally disturbed/mental illness, advanced aged and/or have Alzheimer's/dementia. On 07/07/2026 at 1:20 PM as part of the survey process, Surveyor conducted a review of 3 employee records to verify compliance with task specific training requirements. Surveyor requested training records for Caregiver R, Caregiver T and House Manager S.	N 247			

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N 247	Continued From page 15 The record for Caregiver R, hired 10/03/2025, did not contain evidence of training or evidence of meeting an exemption for activities of daily living or dietary training. The record for Caregiver T, hired 10/08/2026, did not contain evidence of training or evidence of meeting an exemption for activities of daily living or dietary training. The record for House Manager S, hired 08/20/2025, did not contain evidence of training or evidence of meeting an exemption for assessment (to include specific topics of: assessment methodology, assessment of changes in condition, sources of assessment information, and documentation of the assessment), activities of daily living or dietary training. On 07/07/2026 at 1:45 PM, Surveyor interviewed House Manager S. S/he confirmed that caregivers are responsible for performing dietary tasks during their shifts. S/he stated that staff "come with training" if they previously worked in an assisted living facility and that preparing a meal and caring for a resident is "common sense." S/he confirmed the employee files did not contain evidence of training regarding dietary services and activities of daily living for all 3 employees reviewed. During a follow up interview with House Manager S on 07/08/2026 at 10:09 AM, Manager S said his/her training in assessment consisted of shadowing the previous manager for one assessment and then conducting them on his/her own. On 07/13/2026, Licensee A sent an email with 2 documents that included checklists for training.	N 247			

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N 247	Continued From page 16 The documents did not include evidence of a system to ensure caregivers were trained in activities of daily living or dietary training to include determining nutritional needs, menu planning, food preparation and food sanitation. Cross Reference: N0196 DHS 83.14(2)(a) Licensee Ensures Facility Complies with Laws N0230 DHS 83.19 Orientation N0247 DHS 83.22 (1)-(4) Task Specific Training N0381 DHS 83.35(1)(a) Pre-Admission And Ongoing Assessments	N 247		
N 327	83.31(4)(b) Allowable reasons for involuntary discharge. Discharge or transfer initiated by CBRF. Reasons for involuntary discharge. The CBRF may not involuntarily discharge a resident except for any of the following reasons: 1. Nonpayment of charges, following reasonable opportunity to pay. 2. Care is required that is beyond the CBRF 's license classification. 3. Care is required that is inconsistent with the CBRF 's program statement and beyond that which the CBRF is required to provide under the terms of the admission agreement and this chapter. 4. Medical care is required that the CBRF cannot provide. 5. There is imminent risk of serious harm to the health or safety of the resident, other residents or employees, as documented in the resident 's record. 6. As provided under s. 50.03 (5m), Stats. 7. As otherwise permitted by law.	N 327		

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N 327	Continued From page 17 This Rule is not met as evidenced by: Based on record review and interview, the CBRF initiated an involuntary discharge for Resident 12 for reasons inconsistent with the reasons permitted by the code. Findings include: The Department received a complaint about the provider issuing an involuntary discharge for reasons other than those permitted by the code. The provider is licensed to serve up to 8 residents from the following client groups; physically disabled, emotionally disturbed/mental illness, irreversible dementia/Alzheimer's and terminally ill. Department records included a program statement which listed the aforementioned client groups and read in part, "Any of these client groups will be accepted only if there are no behavior problems that would be incompatible with the frail elderly, such as outburst of anger, physical aggression ...We also cannot accept resident who elope on a regular basis ... On 07/07/2026 at 3:00 PM, Surveyor reviewed Resident 12's record including the face sheet, temporary individual service plan (ISP), observation notes and incident reports. Resident 12 was admitted on 12/22/2025, was elderly and had diagnoses to include Alzheimer's. Resident 12's observation notes did not document incidents of outbursts of anger, physical aggressions or persistent exit seeking. The observation notes documented the following incidents of anxiety or nighttime restlessness that were effectively managed with redirection or the	N 327		

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N 327	Continued From page 18 use of prescribed PRN (as needed) medication: 12/25 - "was up all night ...has been resting in [his/her] room since breakfast." 12/26 - "had a good day ...some instances of high anxiety ...easily redirected by music and taking walks ..." 12/28 - "up and down through the night staff kept telling client ...everything is okay [s/he] is safe ..." 12/29 - "seemed agitated and would not stay in bed was redirected several times before given prn after prn ...was in room asleep the rest of the night." 01/07 - "4:45 AM ...pulling on [his/her] doors and becoming very upset staff tried to redirect many times but was unsuccessful PRN was given." The record included an incident report dated 01/12/2026 which documented an elopement incident that occurred around 3:00 AM. Resident 12 was located approximately 3 hours and 45 minutes later by law enforcement. The record included an involuntary discharge notice dated 01/14/2026 which read, "Reasons for the 30-Day Notice: [Resident 12] is going to need more assistance than us as a 1-staff facility cannot [sic] offer [him/her], [s/he] is more of a 2-assist due to the progression of [his/her] dementia." Surveyor interviewed Director W at 10:09 AM. S/he described some instances of anxiety when Resident 12 moved in and explained, "We felt like it was the transition period coming from [his/her] old house where [s/he] had been since a child." Director W said when s/he worked in the facility, Resident 12 would wander within the building but never attempted to leave the facility. When describing Resident 12's adjustment period s/he	N 327		

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N 327	Continued From page 19 said it was "nothing out of the ordinary. I enjoyed working with [him/her]." Surveyor interviewed Manager S on 07/07/2026 at 8:28 AM. S/he provided information about the elopement incident during the overnight hours on 01/12/2026. Manager S said Caregiver R was working that night and at some point discovered Resident 12 was missing. Manager S said their policy is to alarm the doors at 8:00 PM nightly. Manager S said the door alarms are very loud, and if Caregiver R had alarmed them the night of the elopement there would have been "no way" s/he should would not have heard Resident 12 leave. Surveyor interviewed Co-owner K on 07/07/2026 at 11:00 AM. S/he said, "I don't know why [Caregiver R] didn't set the door alarms. [S/he] knows [s/he] is supposed to set them. I think [s/he] fell asleep on shift is what we believe what happened." Surveyor interviewed Licensee A on 07/16/2026 at 1:30 PM. Surveyor noted record review and interviews did not indicate Resident 12 needed the supervision of 2 staff to manage dementia related behaviors and the one elopement incident may have been due to staff negligence to alarm the doors and/or provide adequate supervision. Licensee A acknowledged Surveyor's concerns and said the decision to discharge Resident 12 was a "knee jerk reaction" because the elopement incident was so serious. Cross Reference: N0196 DHS 83.14(2)(a) Licensee Ensures Facility Complies with Laws N0328 DHS 83.31(4)(c) Involuntary Discharge Notice Requirements	N 327		

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{N 328}	83.31(4)(c) Involuntary discharge notice requirements. Discharge or transfer initiated by CBRF. Notice requirements. Every notice of involuntary discharge shall be in writing to the resident or resident ' s legal representative and shall include all of the following: 1. A statement setting forth the reason and justification for discharge listed under par. (b). 2. A statement that the resident or the resident ' s legal representative may ask the department to review the involuntary discharge by sending a written request within 10 days of receipt of the discharge statement to the department ' s regional office with a copy to the CBRF. The notice shall state that the request must provide an explanation why the discharge should not take place. 3. The name, address and telephone number of the department ' s regional office director. 4. The name, address and telephone number of the regional office of the board on aging and long term care ' s ombudsman program. For residents with developmental disability or mental illness, the notice shall include the name, address and telephone number of the protection and advocacy agency designated under s. 51.62(2)(a), Stats. This Rule is not met as evidenced by: Based on record review and interview, the provider did not ensure the written involuntary discharge notice issued to Resident 12 included all required information. This is a repeat deficiency. See Statement of Deficiency UK3M13, dated 07/25/2025. Findings include:	{N 328}		

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{N 328}	Continued From page 21 On 07/07/2026, Surveyor reviewed Resident 12's record which included an involuntary discharge notice dated 01/14/2026. The notice did not include: --A statement that the resident or the resident's legal representative may ask the department to review the involuntary discharge by sending a written request within 10 days of receipt of the discharge statement to the department's regional office with a copy to the CBRF. The notice shall state that the request must provide an explanation why the discharge should not take place. --The name, address and telephone number of the department's regional office director. --The name, address and telephone number of the regional office of the board on aging and long term care's ombudsman program. On 07/15/2026 during the exit conference with Licensee A beginning at 1:30 PM, Surveyor reviewed the findings regarding the content of the involuntary discharge notice issued to Resident 12. Licensee A acknowledged the findings. Cross Reference: N0196 DHS 83.14(2)(a) Licensee Ensures Facility Complies with Laws N0327 DHS 83.31(4)(b) Allowable Reasons For Involuntary Discharge	{N 328}		
N 348	83.32(3)(d) Rights of Residents: Free from mistreatment In addition to the rights under s. 50.09, Stats., each resident shall have all of the following rights: Freedom from mistreatment. Be free from	N 348		

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N 348	Continued From page 22 physical, sexual and mental abuse and neglect, and from financial exploitation and misappropriation of property. This Rule is not met as evidenced by: Based on record review and interview, the provider failed to ensure Resident 12 was free from neglect. According to 46.90(1)(f), neglect is defined as "the failure of a caregiver, as evidenced by an act, omission, or course of conduct, to endeavor to secure or maintain adequate care, services, or supervision for an individual...and creating significant risk or danger to the individual's physical or mental health..." Resident 12 was elderly with diagnoses including Alzheimer's disease. On 01/12/2026 at approximately 3:00 AM, Caregiver R discovered Resident 12 was missing from the facility during outdoor weather conditions of 21°F with 8 mph winds. Caregiver R failed to take immediate action to notify management or law enforcement. Caregiver R first initiated a call to a manager at 4:33 AM, more than 1.5 hours after discovering Resident 12 was missing. S/he subsequently participated in 3 separate phone calls with 2 managers at 4:34 AM, 5:10 AM, and 5:25 AM, and during each call Caregiver R was instructed to call 911. Caregiver R did not call 911, further delaying emergency response efforts. Director W ultimately initiated a call to 911 at 6:18 AM, more than 3 hours after Resident 12's absence was first discovered. Law enforcement located Resident 12 at 6:45 AM on a neighbor's porch with a core body temperature of 93.4°F and a pulse of 56 bpm. The resident was subsequently	N 348		

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N 348	Continued From page 23 hospitalized for the treatment of moderate hypothermia and frostbite affecting all fingertips and both knees. Findings include: The provider is licensed to serve up to 8 residents from the following client groups; physically disabled, emotionally disturbed/mental illness, irreversible dementia/Alzheimer's and terminally ill. The Department received a complaint regarding an elopement incident. The provider submitted a self report to the Department on 01/15/2026 which read in part, "[Caregiver R] was working the NOC (3rd) shift on 01/11, [Resident 12] was stated to be in [his/her] room at 02:00 am, [Caregiver R] stated that [s/he] went to do a room check at 03:00 am and [s/he] stated that [Resident 12] was missing. [s/he] contacted [House Manager S] at 04:30 am...[House Manager S] advised [Caregiver R] to call 911 to activate a missing persons report. [Caregiver R] told [House Manager S] that [s/he] would continue searching the house and [House Manager S] again advised to call 911... Shortly after [Caregiver R] called [House Manager S], [Caregiver R] then called [Director W] who also advised [Caregiver R] to call 911...[Director W] asked [Caregiver R] if [s/he] had checked over the entire house, or if [s/he] went outside to check if [Resident 12] was out in the yard...[Director W] arrived at the facility around 06:15 am, [Caregiver R] denied calling 911 yet, [Director W] immediately called 911 and handed the phone to [Caregiver R] to give the dispatch [Resident 12]'s information. [Director W] then went outside to	N 348		

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N 348	Continued From page 24 look...Shortly after... EMS, and Police arrived at the facility, they advised staff that there was a call of an older [person] wandering around at 03:00 am. They then left to find [Resident 12], it didn't take them too long to find [him/her]. They brought [him/her] back took all of [his/her] information, were warming [his/her] body up as it was cold outside. When...EMT's arrived they took [him/her] up to the ER to get assessed as [s/he] was out in the cold for 3-4 hours and had blood on [his/her] fingers." The report noted Resident 12's body temperature was 90°F, s/he sustained frostbite to his/her fingertips and was hospitalized for treatment. Surveyor reviewed the Wood County Sheriff's Office police report on 07/08/2026: "At approximately 6:18 AM...dispatched to 8679 State Highway 186 in the Town of Arpin...the location of Alice and Louise's Assisted Living Home...advised that [an elderly resident] had last been seen around 2 a.m...While responding, I did hear...there may have been a complaint earlier in the morning that would be associated with this incident...In that complaint [a caller] reported a [person] walking in the road on STH 186 that had gone to a resident one house south of Alice & Louise's...it appeared deputies check the area but were unable to locate anyone...Based on information from that earlier call, units did respond to the house directly south of Alice and Louise's." At 6:45 AM, officers found Resident 12 at the neighbor's front door. On 07/13/2026 at 3:26 PM, Surveyor interviewed Investigator V who stated the initial call about a person walking on the highway came in at 3:15 AM. Surveyor reviewed the paramedics report on 07/09/2026:	N 348		

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N 348	Continued From page 25 "01/12/2026...At 0629 [fire department] was requested to assist in a search for a missing [elderly persons]. Reports indicate pt was last seen at 0200, caller traveling down Hwy 186 reported elderly [person] walking along the road at approx. 0300 just south of that residence. Pt (patient) was located sitting on outside porch of neighbors home. Pt was wearing sweatshirt, thin legging type pants, socks and slippers. Pt was...transported back to assisted living. Pt moved to wheelchair and taken inside. Staff report pt normally confused, doesn't know where [s/he] is at...Pt alert, able to answer simple questions showing some increased confusion compared to baseline. Only complaint is being cold. Shivering present. Wet/damp clothes removed. Pt wrapped in blankets. Pt skin cold, red, peripheral pulses very weak. Pt has lacerations and dry blood on hands/ fingers. Legs appear red, with dark bruised appearance on bilateral knees. Vitals monitored P (pulse) 56...tympanic temp 93.4...Heat packs placed in armpits." Surveyor reviewed historical weather data. On 01/12/2026 at 2:54 AM, the temperature was 21°F and wind speeds were 8 miles per hour. Source: https://www.wunderground.com/history/daily/us/wi/marshfield/KMFI/date/2026-1-12. Retrieval date 07/20/2026. Surveyor reviewed records from the managed care organization (MCO) on 07/10/2026: --Resident 12 had diagnoses to include dementia and a legal guardian. Prior to admission to the facility on 12/22/2025, s/he lived in his/her childhood home. Resident 12 moved into the	N 348		

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N 348	Continued From page 26 facility due to "decline in memory and cognition." --Case note dated 01/18/2026, "Admitted (to hospital) on 1/12 following elopement from CBRF. No definite time of how long [s/he] was outside...being treated for frostbite to all 10 fingertips and bilateral knees. Treated for moderate hypothermia...Noted bradycardia..." --Case note dated 01/20/2026, "Continued treatment to all fingertips and bilateral knees d/t (due to) frostbite." Surveyor interviewed Manager S on 07/07/2026 at 9:28 AM and 07/08/2026 at 10:02 AM: --Manager S said Resident 12 was admitted in late December 2025 with dementia and limited ability to verbally communicate. S/he said Resident 12 was "very anxious in those first few weeks" and at times would wander within the facility. --Manager S recalled the elopement incident and described it as "terrifying." Manager S said s/he missed a call from Caregiver R at 4:33 AM, but answered when Caregiver R called back at 4:34 AM. Caregiver R told Manager S s/he could not find Resident 12 anywhere in the facility and Manager S said s/he instructed Caregiver R to call 911 immediately and reiterated the importance of calling right away because "it's winter, it's cold. I told [Caregiver R] all of that." Manager S said Caregiver R called back at 5:10 AM to report s/he still had not located Resident 12 and s/he still had not called 911. Manager S said s/he again told Caregiver R to call 911 immediately. --Manager S showed Surveyor his/her call log which displayed a missed call from Caregiver R at 4:33 AM, an incoming call lasting 3 minutes at 4:34 AM and an incoming call lasting 2 minutes at 5:10 AM.	N 348		

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N 348	Continued From page 27 --Surveyor asked Manager S if s/he considered calling 911 him/herself, especially after learning Caregiver R did not call between 4:34 AM and 5:10 AM. Manager S replied, "I wish I would have...I had a couple of drinks...I was impaired, so I didn't think of it. [Director W] was on call, I wasn't even supposed to be." Surveyor asked if s/he considered calling Director W or the licensee and s/he reiterated s/he didn't have good judgment at the time s/he received the calls. --Manager S said the doors are supposed to be alarmed after 8:00 PM and demonstrated the door alarms for Surveyor. Surveyor observed they were extremely loud and piercing. Manager S said his/her impression was that Caregiver R either did not alarm the doors, or ignored them. Manager S showed Surveyor a charting system for a task of "door alarms" scheduled daily at 8:00 PM. Surveyor reviewed the month of January 2026, the charting started on 01/16/2026. Manager S confirmed there was no documentation of this nightly task prior to the elopement incident. Manager S also provided Surveyor a document titled "Emergency Missing Resident Protocol" with staff signatures beginning 01/13/2026. Manager S confirmed there was no documentation of staff education on missing resident protocol prior to 01/13/2026. Manager S also said there was no written on-call protocol for caregivers and it was just known that Manager S was on call at all times unless s/he informed caregivers otherwise. Manager S did not have evidence s/he informed caregivers s/he was off duty and not available for calls during the overnight hours 01/11 - 01/12/2026. Surveyor interviewed Director W on 07/07/2026 at 10:02 AM: --Director W recalled the morning of the	N 348		

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N 348	Continued From page 28 elopement and said s/he missed several calls from Caregiver R because his/her phone was on vibrate. Director R showed Surveyor his/her call log and there were missed calls at 5:14 AM, 5:15 AM, 5:16 AM, 5:18 AM and 5:19 AM. At 5:25 AM there was a 7 min outgoing call to Caregiver R. --Director W said, "That was a hard night. I was sleeping. I was supposed to work at 7:00 AM... [Caregiver R] said, I can't find [Resident 12]." Director W said s/he dozed off during the call but eventually realized the seriousness of the situation, instructed Caregiver R to call 911 immediately and then got ready for the day and drove to the facility. Director W said when s/he arrived at the facility around 6:15 AM and realized Caregiver R had not called 911, s/he was shocked and upset. Director W said s/he initiated the call to 911, gave the phone to Caregiver R and then went outside to look for footsteps in the snow. Director W said s/he was very worried about Resident 12 because it was cold out, but Caregiver R kept saying it was not a big deal and everything would be OK. Director W said Caregiver R admitted not turning on the door alarms, claiming s/he didn't know about them. Director W said that was not accurate because at least once prior to the incident s/he arrived for 1st shift, discovered the door alarms were not on and told Caregiver R they needed to be on every night. --Director W said the police found Resident 12 at the house next door and returned [him/her] to the facility while waiting for the ambulance to arrive. Director W said Resident 12 was mumbling, shivering and had blood on his/her fingertips. . --Director W said s/he asked Caregiver R why s/he didn't call 911 as instructed and s/he just kept repeating "everything will be OK" and s/he thought Resident 12 was "hiding in a closet or behind a chair." Director W said s/he could "not	N 348		

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N 348	Continued From page 29 comprehend" why Caregiver R did not call 911 and s/he found the whole situation and Caregiver R's excuses very upsetting. Director W said Caregiver R was suspended immediately and subsequently was fired due to his/her response to the elopement. Surveyor reviewed Resident 12's facility record on 07/08/2026: -S/he was admitted on 12/22/2025. -The provider utilizes ECP, an electronic records software program. Resident 12's record in ECP contained his/her medication administration records, observations notes and individual service plan (ISP). The ISP identified the need for "full assistance" with activities of daily living including dressing, oral care and showering. A section titled "Anxiety - Supervision" read, "Must supervise and monitor any high anxiety behaviors, please keep in mind [s/he] recently moved here and is very nervous as this is [his/her] first facility. Status: To receive complete supervision and constant monitoring and cuing to avoid/prevent self abusive behavior." -Observation note dated 12/23/2025 read, "[Resident 12] has moved in...Please make sure to document things you pick up on about [him/her] to further aid in [his/her] care plan so we can ensure the best care for [him/her]. During morning meds I noticed [s/he] needs cues in order to take [his/her] meds so please make sure you are giving cues and being patient. [S/he] also will not remove [his/her] dishes from the table so make sure you are cleaning up after meals. Per [his/her] guardian here are a few things that we can do when [s/he] is having high anxiety times: Listening to music...Gentle reminders that [s/he] is okay and that [s/he] is safe here..." -Resident 12's medication administration records	N 348		

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N 348	Continued From page 30 and care charting records including charting by Caregiver R, indicating s/he had access to the electronic record. Surveyor and Investigator X from the Office of Caregiver Quality conducted an interview with Caregiver R on 07/14/2026 at 1:50 PM: -Caregiver R said from the first night Resident 12 moved in, s/he did not feel Resident 12 was appropriate for the facility. S/he said, "[Resident 12] has Alzheimer's so bad [she] would walk around pounding things and walls and tried to get in everyone's rooms and muttering. [S/he] had full blown Alzheimer's." Caregiver R said s/he had 15 years experience as a caregiver but never experienced a resident with dementia that was so progressed. -Caregiver R recalled the night of the elopement and said Resident 12 needed his/her constant attention the majority of the night. Caregiver R explained, "[Resident 12] was in high distress, wandering and trying to get into anybody else's room...muttering, banging on things." Caregiver R said, "I spent half the night in [his/her] room just to make sure [s/he] was asleep...you will see [s/he] didn't elope until 3:00 AM." Caregiver R said by that time s/he (Caregiver R) was "worn out" and "tired" from following Resident 12 around all night, but s/he started cleaning tasks in the facility. Caregiver R said subsequently when s/he couldn't find Resident 12, s/he "went into a panic" but convinced him/herself it wasn't possible s/he'd eloped and kept looking for him/her thinking s/he was hiding somewhere. Caregiver R was asked if Resident 12 had ever hidden from him/her before. Caregiver R said Resident 12 would "usually seek me out if I was cleaning or something" and said Resident 12 had never hidden from him/her before.	N 348		

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N 348	Continued From page 31 -Caregiver R placed blame on management and said if Resident 12 was an elopement risk s/he should have been informed. Caregiver R also said management did not inform him/her there were door alarms. -Surveyor asked about the 1.5 hour delay before contacting Manager S and s/he replied, "I was searching all nooks and crannies, under beds and all over the place and I went into shock because there is no way [s/he] could have eloped. I was searching under beds behind shower curtains, I realize now in hindsight I should have done is immediately called 911." Surveyor asked if Manager S informed him/her to call 911 during either of the 2 phone calls at 4:34 AM and 5:10 AM and Caregiver R said s/he could not remember. -Caregiver R said s/he was not proficient with technology, but confirmed s/he had access to resident records in ECP and utilized it to chart medication administration and cares. Surveyor reviewed Resident 12's ISP category titled "Anxiety - Supervision" with Caregiver R and asked if s/he was familiar with it. S/he initially said s/he was but then changed his/her statement to say s/he was only familiar with "the concept" of it. In summary: On 01/12/2026, at approximately 3:00 AM, Caregiver R discovered Resident 12 was not in his/her room. After thoroughly searching the facility, Caregiver R was still unable to locate Resident 12. Despite knowing that Resident 12 was highly vulnerable due to his/her diagnoses, and that the outdoor weather conditions were dangerous at 21°F with 8 mph winds, Caregiver R failed to immediately contact 911 or facility management. A total of 1.5 hours passed before Caregiver R notified management. Between 4:34 AM and 5:25 AM, Caregiver R participated in	N 348		

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N 348	Continued From page 32 three phone calls with two different managers, both of whom instructed Caregiver R to call 911. At 6:15 AM Director W arrived at the facility, discovered that Caregiver R had still not called 911 and initiated the emergency call him/herself. Resident 12 was located by law enforcement at 6:45 AM on a neighbor's porch. Upon discovery, Resident 12's heart rate was 56 bpm, his/her body temperature was 93.4°F, and his/her fingers were bleeding. Resident 12 was subsequently admitted to the hospital for moderate hypothermia and the treatment of frostbite to all fingertips and both knees. Cross Reference: N0196 DHS 83.14(2)(a) Licensee Ensures Facility Complies with Laws -	N 348		
{N 352}	83.32(3)(h) Rights of Residents: Receive medication In addition to the rights under s. 50.09, Stats., each resident shall have all of the following rights: Receive medication. Receive all prescribed medications in the dosage and at intervals prescribed by a practitioner. The resident has the right to refuse medication unless the medication is court ordered. This Rule is not met as evidenced by: Based on record review and interview, the provider did not ensure Resident 11 received all prescribed medications in the dosage and intervals prescribed by a practitioner. The provider did not ensure refills for 2 of Resident	{N 352}		

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{N 352}	Continued From page 33 11's prescribed medications on 06/07/2026 resulting in missed doses and on 06/14/2026 administered a different resident's medications to Resident 11. The is a repeat deficiency being cited for the fourth time. See Statement of Deficiency (SOD) UK3M11, dated 09/11/2023; SOD UK3M12, dated 07/01/2024; SOD UK3M13, dated 07/25/2025. Findings include: EXAMPLE 1: On 07/08/2026 at 11:15 AM, Surveyor reviewed Resident 11's record. -Resident 11 was admitted on 08/01/2024 and his/her current individual service plan identified the need for staff assistance with medication administration. -An incident report dated 06/14/2026 read, "Medication - Error ...staff went into residents [sic] room and gave 8pm meds and gave the wrong med to resident ...Physician Notified? No" The report documented Resident 11 was administered his/her own scheduled medications at 7:38 PM as well to include hydralazine 10 mg (blood pressure medication), atorvastatin 40 mg (cholesterol medication), melatonin 6 mg (for sleep) and famotidine 20 mg (acid reflux medication). Author: Caregiver Y -Surveyor noted the incident report did not identify the medications that were administered in error to Resident 11. On 07/08/2026 at 12:08 PM, Surveyor interviewed Manager S about the medication error. Manager S said s/he would need to look through his/her text messages with Caregiver Y to refresh his/her memory and confirmed the	{N 352}			

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{N 352}	Continued From page 34 information was not located anywhere else in the records. Manager S looked through texts and said s/he couldn't find the information but recalled it was Resident 6's medication that was administered in error and specifically recalled lorazepam. While looking through his/her phone, Manager S said s/he found a 4 min call with Caregiver Y in the call log but added, "It's been like a month. I don't remember what was said on the phone call." Manager S said the physician was not notified of the medication error but s/he was certain s/he told Caregiver Y to monitor Resident 11 and if there would be a change in condition they would need to have him/her seen by a physician. On 07/08/2026 at 12:42 PM, Surveyor reviewed Resident 6's medication administration record (MAR) and noted s/he had a total of 4 medications scheduled for 8:00 PM; hydralazine 25 mg (blood pressure medication), lorazepam 1 mg (anxiety medication), metoprolol tartrate 25 mg (blood pressure medication) and olanzapine 5 mg (anti-psychotic). During the interview with Manager S, s/he said Caregiver Y had not yet been officially retrained but after the medication error s/he observed Caregiver Y for 3 medication passes. Manager S said there was no documentation of the times s/he observed Caregiver Y. Surveyor audited the medication cart at 12:28 PM, noting Caregiver Y had been the medication passer earlier in the morning. Surveyor observed Resident 10 had 11 medications scheduled for 8:00 AM and 11 of 11 medications remained in the cards. Resident 10's MAR contained no documentation about why the medications weren't administered. Manager S said Caregiver	{N 352}		

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{N 352}	Continued From page 35 Y verbally reported to Manager S earlier in the day that Resident 10 refused the medications. Manager S confirmed Caregiver Y did not document the omission in the record or the reason why the medications were not administered prior to his/her shift ending at approximately 9:30 AM. EXAMPLE 2: During the aforementioned review of Resident 11's record, Surveyor reviewed the June 2026 MAR. On 06/07/2026, aspirin (82 mg) scheduled once daily and loratadine (10 mg) scheduled once daily, were not administered because the provider was "out of the medication." Surveyor conducted an interview with Manager S and Director W on 07/08/2026 at 1:53 PM. The managers confirmed Resident 11 was prescribed the 2 medications and did not receive them as ordered on 06/07/2026. Manager S said each month the medication cycle begins on the 7th and the medications are delivered 7-10 days prior. During June, Resident 11's aspirin and loratadine were not filled or delivered with the cycle because his/her physician needed to write an updated prescription. Manager S said s/he called the pharmacy "every single day" but did not document any of the calls and said s/he would have to "document every day all the time if I documented all the times I called the pharmacy for residents." Manager S also placed blame on the physician for not being responsive. Manager S did not recall how s/he was effective in ensuring the medications were available the day after they ran out. Director W suggested to Manager S that in the future, s/he could monitor the medication cards each month which will indicate how many refills are left, affording him/her the opportunity to	{N 352}		

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{N 352}	Continued From page 36 resolve any refill complications early and before refills run out. Cross Reference: N0196 DHS 83.14(2)(a) Licensee Ensures Facility Complies with Laws	{N 352}			
{N 355}	83.32(3)(k) Rights of Residents: Self-determination In addition to the rights under s. 50.09, Stats., each resident shall have all of the following rights: Self-determination. Make decisions relating to care, activities, daily routines and other aspects of life which enhance the resident ' s self reliance and support the resident ' s autonomy and decision making. This Rule is not met as evidenced by: Based on record review, observation and interview, the provider did not ensure residents were able to make decisions relating to food and snacks to support residents' autonomy and decision making. Caregiver Y restricted Resident 6 and other residents' access and choice in snacks. This is a repeat deficiency being cited for the third time. See Statement of Deficiency (SOD) UK3M12, dated 07/01/2024 and SOD UK3M13, dated 07/25/2025. Findings include: On 07/07/2026 at 9:28 AM, Surveyor conducted an interview with Manager S and asked if any residents had restrictions on food. Manager S	{N 355}			

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{N 355}	Continued From page 37 said Resident 6 does not have restrictions, but his/her candy is kept in the office because his/her doctor wants his/her consumption of snacks monitored. Manager S reiterated there are no restrictions and when Resident 6 asks for his/her candy, it is given to him/her. Surveyor observed a cabinet in the office with a shelf labeled for Resident 6 and it contained 2 bags of chips, nuts and diet soda. Surveyor reviewed Resident 6's record on 07/07/2026 at 3:40 PM. Resident 6's individual service plan read, "Diabetic Snack Monitor: Monitor what [s/he] is having for a snack. [His/her] doctor is wanting to monitor [his/her] sugar intake to see what is causing the inconsistencies with [his/her] blood sugars in the mornings ...04/06/2026." Surveyor reviewed Resident 6's record for documentation of blood sugar values between 06/03 - 06/15/2026 and noted the morning checks were within a range of 82-119 mg/dl. Surveyor reviewed Resident 6's observation notes and noted Caregiver Y restricted Resident 6's choice of snacks on at least two dates: 06/12/2026: "[Resident 6] was in the island cabinets grabbing sweets, I then ...redirected [him/her] and told [him/her] [s/he] could have a healthy snack if [s/he] was still hungry. Staff then notified [Manager S] of the behavior due to [Resident 6] having blood sugar checks due to being diabetic in case of a change in [his/her] normally steady number..." 06/13/2026 "[Resident 6]...went to [his/her] room around 11pm tonight and staff heard [Resident 6]	{N 355}		

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{N 355}	Continued From page 38 eating something in [his/her] room and knocked on the door and saw [his/her] hiding the wrappers in [his/her] garbage can and staff asked what [s/he] was eating and [Resident 6] then stated nothing and Staff then redirected and stated to resident do not lie to staff because you have blood sugars to be taken in the morning and you had snack already. Staff then redirected resident to open [his/her] garbage can in [his/her] room and resident did and staff found 4 wrappers of peanut butter crackers in [Resident 6]'s garbage can. Staff then called the boss and had a talk with [Resident 6] over the phone tonight about how [Resident 6] cannot be taking stuff out of the cabinets at almost midnight. [Resident 6] then stated to boss I won't do it anymore..." On 07/07/2026 at 4:27 PM, Surveyor interviewed Resident 6. S/he said s/he can only have snacks when they are scheduled at 2:00 PM and 8:00 PM, and said staff will "yell at me" if s/he tries to have snacks at other times. On 07/07/2026, Surveyor conducted a return visit to the facility at 8:25 PM. Upon arrival, Surveyor observed Caregiver Y outside taking a break. Surveyor and Caregiver Y entered the facility and at 8:30 PM, Surveyor observed Resident 6 request a snack. Caregiver Y offered Resident 6 a banana and s/he accepted. Caregiver Y said snack normally is offered at 8:00 PM, but s/he was busy so it hadn't been offered it yet. Surveyor asked if there are any limitations on the type of evening snacks residents are allowed to have. Caregiver Y said, "Yes, [Resident 6] has limits on snacks, everyone does. It is required they have fruit at night. No sweets or nothing. [Resident 6] is a diabetic, so [s/he] can not have any sugar at night...[Resident 6] will sneak candy and snacks. If I'm doing something I'll peek my head and look.	{N 355}		

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{N 355}	Continued From page 39 One day I caught [him/her] in the cabinet and one of the doors squeaks. I heard it slam and heard the chip bag. I'm like, 'What are you doing? You know you can't have that!' [Resident 6] apologized, but I called [Manager S] and [Resident 6] will get talked to by [Manager S] in the morning." Surveyor asked if Resident 6's blood sugars have been too high in the morning and Caregiver Y said s/he had no idea because the day shift tests his/her blood sugars. On 07/08/2026 at 10:09 AM, Surveyor interviewed Manager S about the observations and interview with Caregiver Y. Manager S said s/he felt it was appropriate to limit snacks to fruits during the evening snack because the code requires a nutritious snack be offered twice per day. Surveyor provided information that a nutritious snack can be offered, while still respecting residents' rights to have choice. Surveyor interviewed Licensee A on 07/15/2026 at 1:30 PM and reviewed the aforementioned findings. Licensee A expressed frustration that in spite of staff training, some staff continue to struggle with respecting residents' rights to exercise choice. Licensee A said s/he tries to audit the observation notes regularly, but could not recall if s/he saw the June 2026 notes authored by Caregiver Y or if/how s/he responded. Cross Reference: N0196 DHS 83.14(2)(a) Licensee Ensures Facility Complies with Laws	{N 355}		
{N 381}	83.35(1)(a) Pre-admission and ongoing assessments.	{N 381}		

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{N 381}	Continued From page 40 Scope. The CBRF shall assess each resident ' s needs, abilities, and physical and mental condition before admitting the person to the CBRF, when there is a change in needs, abilities or condition, and at least annually. The assessment shall include all areas listed under par. (c). This requirement includes individuals receiving respite care in the CBRF. For emergency admissions the CBRF shall conduct the assessment within 5 days after admission. This Rule is not met as evidenced by: Based on record review and interview, the provider conducted a preadmission assessment for Resident 13 at the hospital on 05/01/2026 that did not include all required areas of assessment and did not capture his/her documented complex medical diagnoses, behaviors or all prescribed medication. After the assessment and prior to the anticipated admission, Resident 13 sustained a fall and hip fracture and was admitted to an inpatient rehabilitation facility (IRF). The rehabilitation facility documented pronounced behaviors and related medication changes. When Resident 13 was ready for discharge from the IRF, the provider conducted a virtual assessment but did not document the assessment. Resident 13 was admitted to the facility on 06/05/2026. S/he displayed behaviors and experienced falls. Staff were unable to safely meet his/her needs for transfer assistance and fall prevention. After the fourth fall, s/he was admitted to the hospital and did not return because his/her needs exceeded the capacity of the facility. This is a repeat deficiency. See Statement of	{N 381}		

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{N 381}	Continued From page 41 Deficiency UK3M13, dated 07/25/2026. Findings include: On 07/07/2026 at 9:00 AM and 3:00 PM, Surveyor interviewed Manager S. Manager S said Resident 13 was admitted on Friday 06/05/2026. During the first weekend, s/he refused medication, refused cares, exhibited behaviors and had 6 falls. Manager S said, "After the final fall...decided to send [him/her] to the hospital. They said [s/he] had a urinary tract infection and a foot infection, we had no idea. [S/he] had been here 3 or 4 days. [The managed care organization] and I talked last week about [Resident 13] not coming back...[s/he] needs skilled nursing." Surveyor asked about the pre-admission assessment process for Resident 13. Manager S said s/he assessed Resident 13 while at the hospital on 05/01/2026. After the assessment and while still hospitalized, Resident 13 sustained a fall resulting in a fractured hip so the admission was canceled and s/he was admitted to an IRF. Subsequently, there was another request to consider admission so Manager S said s/he assessed Resident 13 virtually and agreed to admit Resident 13. Manager S said s/he only documented the initial assessment on 05/01/2026 and did not document the subsequent virtual assessment. Surveyor reviewed documentation of the assessment on 05/01/2026 and the IRF discharge summary (both provided by Manager S on 07/08/2026) and noted the following: The 05/01/2026 assessment document only listed 1 diagnoses; type 1 diabetes and a note that	{N 381}		

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{N 381}	Continued From page 42 read, "verbalized depression." Three medications were listed; insulin, tramadol and gabapentin. The assessment noted "slight" tendencies for verbal abuse and no indications of aggression. The assessment noted no pain was present. In contrast, Surveyor noted the IRF discharge summary dated 06/05/2026, detailed complex medical diagnoses, significant behavior management needs, pain management needs and skin integrity concerns. The summary read in part (quoted as written): "This is [an individual with a recent] prolonged hospitalization...from 2/6/2026 through 05/21/2026...relevant past medical history of subdural hematoma after motor vehicle accident, poorly-controlled type 1 diabetes, essential hypertension depression with anxiety left footdrop alcohol dependence with withdrawal hyperglycemia tobacco use and rhabdomyolysis. (Surveyor note: rhabdomyolysis - rare muscle injury involving rapid breakdown of damaged skeletal muscle.)...presented to [hospital] on 02/06/2026 due generalized weakness was found to have profound hypoglycemia with indications of diabetic ketoacidosis. During [his/her] stay developed left scrotal...buttock abscess secondary to orchitis with neurogenic bladder and urinary retention complicating such....was treated for severe protein calorie malnutrition which apparently was in the setting of [his/her] is cognitive impairment due to [his/her] brain injury and as a result [s/he] was treated...with trial of mirtazapine nutrition consultation and increased protein intake...did have some acute blood loss anemia on 05/07/2026 in the setting of a left hip fracture...on 5/4/5 due to vasovagal episodes. (Surveyor note: vasovagal episodes - temporary drop in heart rate and blood pressure causing	{N 381}		

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{N 381}	Continued From page 43 sudden fainting.) [S/he] is noted to undergo closed reduction of the left intertrochanteric hip fracture....extensive time was spent discussing the patient's condition with the patient's [sibling]. We discussed the patient's agitation and refusal to participate (and) [his/her] violent behavior. We discussed placement in a subacute nursing facility or behavioral center..We discussed the use of Seroquel...[sibling] was agreeable to increasing the dose of Seroquel to titrate to improve behavior and interaction...patient was accepted for transfer to the post acute Medical acute inpatient rehabilitation hospital... -May 27th...clear improvement in the patient's behavior...increasing dose of Seroquel as needed to control behavior and improve participation. Increasing evening dose to 200 mg. -May 28...continues to have some behaviors in the morning...responding well to Seroquel and needs an increasing dose...will increase to 300 mg of Seroquel at HS (bedtime) and 200 mg in the morning. -May 29 improved behavior at Seroquel 300 mg at hs and 200 in the morning continue this dose for now... -May 30th...overall behaviors improved Seroquel appears to be helping however continues to have...violent behaviors in the morning. Increasing Seroquel to 300 mg p.o. b.i.d. observe for affect -May 31st behaviors are much improved participation in therapy is much improved continue current dose -June 1st still with inappropriate confused behaviors Seroquel is helping dampen [his/her] acting-out and violence but continues with inappropriate confused behaviors. -June 2nd generally the patient appears to be overall improved on Seroquel...violent behaviors	{N 381}			

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{N 381}	Continued From page 44 have significantly reduced...remains a very challenging placement...due to [his/her] behaviors. For this reason I am increasing the Seroquel again today to 400 mg p.o. b.i.d. this is the maximum dose I will go to. -June 3rd currently on Seroquel 400 mg p.o. b.i.d. generally the patient is more engaged less combative fewer behaviors better participation but by no means adequate -Urinary tract infection: Continue Keflex through 6/4... Discharge planning .. -June 2nd it is acknowledge that this patient is making very little progress. [S/he] continues to require placement in an appropriate facility for [his/her] traumatic brain injury needs. [His/her] violent behavior has been a barrier to placement. [S/he] is somewhat improved on Seroquel...doses now increased to the maximum 400 mg p.o. b.i.d. Severe hypertension managed as per Internal Medicine...Internal Medicine is consulted to follow for assistance with management of complex medical issues during the IRF stay...pain management- a multimodal pain program is implemented to facilitate patient understanding of the nature of the pain and to focus on independent active physical steps including stretching and repositioning for pain relief...will continue to follow for changes in pain complaints and adjust pain medication as appropriate to achieve adequate relief for participation in therapies...wound care/skin-rehabilitation nursing will maintain routine skin care precautions to avoid excess moisture and skin maceration...with frequent position changes and reminders..." Surveyor reviewed Resident 13's observation notes on 07/08/2026:	{N 381}		

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{N 381}	Continued From page 45 06/05: Admitted to facility. Yelled and swore at staff after learning there was no call button. "It took both staff to transfer [Resident 6] to bed and [Resident 6] did not help at all and [his/her] knees kept buckling under [him/her]..also could not sit up without almost falling over..." 06/06: "couldn't transfer safely to [his/her] wheelchair for supper...couldn't sit up for more than 20 seconds at a time and continued to fall back on bed...staff stayed in [his/her] room the entire time...so [s/he] wouldn't choke when [s/he] kept tipping over...[Resident 6] would also yell if Staff tried to hold [him/her] up..." 06/06: unwitnessed fall, fire department called to help transfer back to bed 06/07: 2:00 PM had difficulty during lunch "could not sit up without assistance and kept falling back on to bed...couldn't transfer safely with one staff." 06/07: 5:45 PM "walked to the bathroom with assistance and almost fell and kept leaning to [his/her] left side...almost missed the toilet." Screamed at staff who were trying to assist. While walking back to bed "knees were buckling" 06/07: 10:00 PM "fell out of bed..." 06/08: 3:15 AM "another unwitnessed fall in [his/her] room with EMS having to be called. EMS suggested calling an ambulance next time to get [him/her] reevaluated as this is now a safety concern..." 06/08: 9:45 AM "scooting to end of bed...staff tried to scoot [him/her] back...staff stepped outside to call supervisor to ask what the next course of action should be, when staff went back inside a few minutes later resident was now on the floor...called ambulance...taken to [hospital]." In conclusion: The provider did not conduct and document an accurate preadmission assessment. The initial assessment on 05/01/2026 omitted Resident 13 's extensive	{N 381}		

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{N 381}	Continued From page 46 diagnoses, behaviors, and medications. Following the resident's subsequent hip fracture and rehabilitation stay, the provider conducted a virtual assessment but did not document it. Resident 13 was admitted on 06/05/2026 for a stay that lasted only three days. During this time, the resident refused care, exhibited behaviors, experienced at least four falls, and could not be safely transferred by staff. On 06/08/2026, the resident was hospitalized, and it was determined that their needs exceeded the facility's capacity. Cross Reference: N0196 DHS 83.14(2)(a) Licensee Ensures Facility Complies with Laws N0385 DHS 83.35(2) Temporary Service Plan	{N 381}		
{N 385}	83.35(2) Temporary Service Plan. Temporary service plan. Upon admission, the CBRF shall prepare and implement a written temporary service plan to meet the immediate needs of the resident, including persons admitted for respite care, until the individual service plan under sub. (3) is developed and implemented. This Rule is not met as evidenced by: Based on record review and interview, the provider did not develop a temporary individual service plan (ISP) based on assessment to meet the immediate needs of Resident 13. This is a repeat deficiency. See Statement of Deficiency UK3M13, dated 07/25/2025. Findings include: Surveyor reviewed Resident 13's record on 07/08/2026 at 9:40 AM.	{N 385}		

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{N 385}	Continued From page 47 The record contained an inpatient rehabilitation facility (IRF) discharge summary dated 06/05/2026. The IRF documented the following: -combative and violent behaviors -diagnoses of traumatic brain injury and severe calorie malnutrition -fall with hip fracture on 05/04/2026 -skin integrity concerns Resident 13's record indicated s/he was admitted to the facility on 06/05/2026. The record contained a temporary ISP dated 06/03/2026, however the ISP did not identify how the provider would meet the needs of the resident upon admission in the following areas: -The ISP was silent to diagnoses of traumatic brain injury and severe calorie malnutrition or needs associated with the diagnoses. -The ISP was silent to transfer needs, ambulation status or fall prevention. -The ISP included inaccurate information regarding behaviors. It read, "Aggressive/Combative...displays no aggressive/combative behaviors." On 07/08/2026 at 1:53 PM, Surveyor reviewed the aforementioned concerns with Manager S and Co-Owner K. They acknowledged Surveyor's review of the record. Cross Reference: N0196 DHS 83.14(2)(a) Licensee Ensures Facility Complies with Laws N0381 DHS 83.35(1)(a) Pre-Admission And Ongoing Assessments	{N 385}		

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{N 387}	Continued From page 48	{N 387}		
{N 387}	83.35(3)(b) Service plan development: parties involved Development. The CBRF shall involve the resident and the resident ' s legal representative, as appropriate, in developing the individual service plan and the resident or the resident ' s legal representative shall sign the plan acknowledging their involvement in, understanding of and agreement with the plan. If a resident has a medical prognosis of terminal illness, a hospice program or home health care agency, as identified in s. HFS 83.38(2) shall, in cooperation with the CBRF, coordinate the development of the individual service plan and its approval under s. HFS 83.38 (2) (b). The resident ' s case manager, if any, and any health care providers, shall be invited to participate in the development of the service plan. This Rule is not met as evidenced by: Based on record review and interview, the provider did not ensure Resident 6's legal representative signed his/her individual service plan (ISP), acknowledging their involvement in, understanding of and agreement with the plan. This is a repeat deficiency. See Statement of Deficiency UK3M13, dated 07/25/2026. Findings include: On 07/08/2026, Surveyor reviewed portions of Resident 6's record to verify correction from the previous deficiency and did not locate evidence of a signed/dated ISP. At 10:10 AM, Surveyor requested Manager S provide Resident 6's most	{N 387}		

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{N 387}	Continued From page 49 current signed and dated ISP. On 07/08/2026 at 2:00 PM, Manager S informed Surveyor s/he did not have a copy of any ISP signed by Resident 6's legal guardian. Manager S said Resident 6's guardian is not very involved or responsive which has presented challenges. Manager S did not provide evidence of attempts to involve or obtain a signature from Resident 6's guardian since the last survey in July 2025. Cross Reference: N0196 DHS 83.14(2)(a) Licensee Ensures Facility Complies with Laws	{N 387}		
{N 389}	83.35(3)(d) Service plans updated annually or on changes Individual service plan review. Annually or when there is a change in a resident ' s needs, abilities or physical or mental condition, the individual service plan shall be reviewed and revised based on the assessment under sub. (1). All reviews of the individual service plan shall include input from the resident or legal representative, case manager, resident care staff, and other service providers as appropriate. The resident or resident ' s legal representative shall sign the individual service plan, acknowledging their involvement in, understanding of and agreement with the individual service plan. This Rule is not met as evidenced by: Based on record review and interview, the provider did not ensure Resident 10 and Resident 11's individual service plans were updated based on assessment to identify changes in their needs	{N 389}		

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{N 389}	Continued From page 50 and associated interventions. Resident 10's ISP was not updated based on assessment to identify comprehensive supervision needs for elopement prevention. Resident 11's ISP was not updated based on assessment to identify his/her needs for fall prevention. This is a repeat deficiency being cited for the third time. See Statement of Deficiency (SOD) UK3M11, dated 09/11/2023 and SOD UK3M13, dated 07/25/2025. Findings include: RESIDENT 10 Surveyor interviewed Co-Owner K on 07/07/2026 at 11:00 AM. S/he stated the 2 exit doors have alarms which are to be activated nightly 8:00 PM, in part because staff need to be alerted when Resident 10 goes outside to smoke. Co-Owner K also stated Resident 10 has experienced changes in his/her cognition and recently made comments indicating s/he may be at risk for elopement. Co-Owner K said the comments have increased because Resident 10 is upset that the guardian and managed care organization (MCO) want to move Resident 10 due to his/her increased supervision needs. Surveyor interviewed Director W on 07/07/2026 at 1:10 PM. S/he also stated the door alarms are to be activated nightly at 8:00 PM primarily to alert staff when Resident 10 goes outside to smoke. Surveyor interviewed Manager S on 07/08/2026 at 10:10 AM. Manager S said, "[The managed	{N 389}		

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{N 389}	Continued From page 51 care organization (MCO)] wants the doors alarmed at night because they think [s/he] is going to wander with his/her recent increase in delusions and hallucinations." Surveyor observed the door alarms were not used during the day when Surveyor was onsite 07/07/2026 and 07/08/2026 or during an evening visit on 07/07/2026 at 8:25 PM. During the 07/07/2026 evening visit, Surveyor interviewed Caregiver Y. S/he acknowledged the doors were not alarmed upon Surveyor's arrival and s/he "sometimes deactivates it a little at night" because otherwise the alarm is disturbing to residents because both s/he and Resident 10 go outside to smoke. Caregiver Y said, "Resident 10 is an elopement risk, but [s/he] has never left the building at all." Caregiver Y said Resident 10 has hallucinations involving giraffes that are outside, but s/he did not describe strategies to supervise Resident 10 when s/he experienced hallucinations or supervision when s/he is outside smoking and said, "S/he doesn't need anyone to go out with [him/her]. [S/he] does it independently." Surveyor interviewed MCO Care Coach Z on 07/14/2026 at 11:30 AM. S/he confirmed Resident 10 has increased supervision needs connected to progression in his/her dementia. Care Coach Z said sometimes Resident 10 has hallucinations and thinks s/he sees giraffes outside, and then wants to go pet them. Care Coach Z said both s/he and Resident 10's guardian have concerns about the level of supervision at the facility and it is a contributing factor to the guardian's decision to move Resident 10. Care Coach Z said Resident 10 is upset about the potential move and if the	{N 389}			

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{N 389}	Continued From page 52 provider were to ensure increased supervision, there is the potential Resident 10 could stay. However Care Coach Z was not optimistic the provider would make necessary adjustments. Surveyor received and reviewed records from the MCO on 07/10/2026. A visit note on 05/08/2026 read, "presented with increased forgetfulness...worsening of [his/her] short term memory..." The member centered plan dated 05/08/2026, documented a diagnoses of anxiety, major depressive disorder and unspecified dementia. "alcohol induced persisting amnesic disorder." Surveyor reviewed portions of Resident 10's facility file including his/her face sheet and current individual service plan. Resident 10 was admitted on 07/12/2021. An ISP update on 06/02/2026 read, "requires the door alarms to be on during the day due to progression in the dementia causing [him/her] to visualize objects that are not there and starting to talk about wandering...requires 10 minute checks when [s/he] is outside of the facility, these are to ensure [his/her] safety." -The ISP was not updated to identify specific hallucinations that may make Resident 10 prone to elopement or interventions staff should use when Resident 10 was experiencing hallucinations. -The ISP was not updated to identify supervision needs other than when Resident 10 was outside. -The ISP was inconsistent with interviews which identified the need for doors to be alarmed at night. Surveyor interviewed Manager S and Co-Owner K on 07/08/2026 at 1:53 PM. Surveyor reviewed	{N 389}		

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{N 389}	Continued From page 53 the aforementioned concerns with the ISP and observations about use of door alarms. The managers acknowledged Surveyor's concerns. RESIDENT 11 Surveyor reviewed Resident 11's record on 07/08/2026 at 11:15 AM. Resident 11 was admitted on 08/01/2024. Resident 11 experienced falls on 11/13/2025 (unwitnessed), 11/18/2025 (unwitnessed), 12/02/2025 (unwitnessed), 12/18/2025 (unwitnessed), 01/06/2026 (witnessed) and 01/26/2026 (unwitnessed). The incident report for the 01/26/2026 fall read, "3:20 AM...I heard resident crying out for someone to come help...was trying to get into [his/her] wheelchair from bed...slipped...hit the floor...Non-emergency services were called to help assist [him/her] back into bed...[s/he] stated...'it's just these socks that made this floor so slippery that I slid out of bed!'" On 01/30/2026, Resident 11's current ISP was updated regarding falls and read, "Fall Risk...has been having frequent falls recently. Staff will assist with [his/her] transfers as [s/he] will allow. Document each occurrence [sic] of a fall immediately to management." On 03/24/2026 at 12:45 AM, Resident 11 experienced another unwitnessed fall. Resident 11 fell when trying to get up to use the bathroom. Non-emergency was called to transfer him/her off the floor and into his/her wheelchair. Resident 11's ISP was not updated after the 03/24/2026 fall. In addition, the previous update on 01/30/2026 did not identify specific strategies	{N 389}			

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{N 389}	Continued From page 54 for encouraging transfer assistance, interventions to pro-actively offer assistance, increased supervision or ensuring proper footwear. On 07/08/2026 at 1:53 PM, Surveyor interviewed Manager S and Co-Owner K. They acknowledged Surveyor's review of the record and indicated they did not have a process to conduct assessments after falls to determine if new or revised interventions were warranted. Manager S said they did implement a change involving a new bed for Resident 11, however s/he acknowledged the change was not identified in the ISP. According to the DHS Publication titled "Falls In Assisted Living Facilities", revised 02/2022, "Falls resulting in serious injury or death continue to occur in assisted living settings. For this reason, the Division of Quality Assurance (DQA) is reminding assisted living providers of the need to identify residents at risk for falls and to implement strategies for falls prevention...Once risks are identified, the resident's individual service plan (ISP) should contain approaches to prevent falls from occurring. The following are some examples of interventions...-Teaching correct use of assistive devices - Environmental modifications, such as lighting, location of furniture, clear pathways, low bed - Daily schedule modifications - Anticipation of needs, including position changes and ambulation, toileting, food or fluids - Physical assistance with daily living activities - Supervision - Proper footwear...Every fall should result in a post-fall assessment to determine the cause of the fall and the need for additional or modified interventions. Comprehensive reporting information will identify date, time, location, a description of what the individual was doing, assistive devices in use and their condition, environmental status, and physical or mental	{N 389}			

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{N 389}	Continued From page 55 condition of the person prior to the fall. Review of those assessments can also reveal a pattern associated with time, place, and staff present. The post-fall assessment may generate the following examples of information that could have contributed to the fall: -Lack of adequate supervision or staff assistance...Worn out, inappropriate, or ill-fitting footwear..." Cross Reference: N0196 DHS 83.14(2)(a) Licensee Ensures Facility Complies with Laws	{N 389}		
{N 431}	83.38(1)(g) Health monitoring. As appropriate, the CBRF shall teach residents the necessary skills to achieve and maintain the resident ' s highest level of functioning. In addition to the assessed needs as determined under s. DHS 83.35(1), the CBRF shall provide or arrange services adequate to meet the needs of the residents in all of the following areas: Health monitoring. 1. The CBRF shall monitor the health of residents and make arrangements for physical health, oral health or mental health services unless otherwise arranged for by the resident. Each resident shall have an annual physical health examination completed by a physician or an advanced practice nurse as defined in s. N 8.02(1), unless seen by a physician or an advanced practice nurse as defined in s. N 8.02(1) more frequently. 2. When indicated, a CBRF shall observe residents ' food and fluid intake and acceptance of diet. The CBRF shall report significant deviations from normal food and fluid intake patterns to the resident ' s physician or dietician. 3. The CBRF shall document communication with the resident ' s physician and other health care providers, and shall record any	{N 431}		

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{N 431}	Continued From page 56 changes in the resident ' s health or mental health status in the resident ' s record. This Rule is not met as evidenced by: Based on observation, record review, and interview, the facility did not notify Resident 11 ' s physician after the resident sustained a heel injury resulting in a 1.5-inch open wound. Four days post-injury, on 07/06/2026, the Managed Care Organization (MCO) was informed during a routine visit, notified the physician, and requested a home health wound care referral. As of 07/13/2026, the resident still had not been evaluated by home health and the facility did not inform the MCO of the delay. The delay in medical assessment and treatment placed Resident 11 at risk due to his/her history of skin integrity concerns and prior hospitalizations for skin infections. This is a repeat deficiency being cited for the third time. See Statement of Deficiency (SOD) UK3M11, dated 09/11/2023 and SOD UK3M13, dated 07/25/2025. Findings include: On 07/07/2026 at 8:45 PM, Surveyor observed Resident 11 ambulating independently with a wheelchair. Resident 11 volunteered s/he had an "Achilles heel injury" caused by his/her bed springs and it was painful. On 07/08/2026, at 11:14 AM, Surveyor reviewed Resident 11's record. S/he was admitted on 08/01/2024 with a history of strokes resulting in body weakness. The individual service plan, current as of the day of the visit, was silent to the	{N 431}			

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{N 431}	Continued From page 57 reported injury or skin integrity concerns. However, the observation notes identified recent injuries to Resident 11's foot. --06/29/2026: "caught [his/her] foot on the springs of [his/her] bed frame, there is a small cut on the back of [his/her] heel. Continue to watch it." Author: Manager S --07/02/2026: "did scrape open the back of [his/her] foot. Management notified and bandage on [his/her] foot." Author: Caregiver AA On 07/08/2026 at 11:57 AM, Surveyor interviewed Resident 11 in his/her room. Surveyor observed a bandage on Resident 11's left heel. Resident 11 demonstrated how s/he injured his/her foot while getting out of bed and catching his/her heel on the metal bed springs. Surveyor observed approximately 3 inches of the metal bed frame and springs were exposed. Resident 11 said s/he recently healed from a different injury on his/her foot that was infected and required medical treatment. At 12:00 PM, Surveyor observed Director W removed the bandage which was covering a circular, open wound approximately 1.5 inches in diameter. Resident 11 said the area "throbs and hurts." Surveyor observed Resident 11's lower extremities, noting bilateral edema in the feet and ankles. The skin over these areas appeared thin with visible scaling. Surveyor asked if the wound was medically assessed and Resident 11 said s/he did not want to go out to see a doctor but s/he was willing to work with home health again. Manager S clarified that Resident 11 was seen by home health from January 2026 - June 2026 for an infected wound on his/her foot and another referral was in the	{N 431}		

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{N 431}	Continued From page 58 works. Surveyor conducted a follow up interview with Manager S about Resident 11's wound. S/he explained Resident 11's injury on 06/29 was not significant and the injury observed today was sustained on 07/02/2026. Manager S said s/he learned of the 07/02 injury the day it occurred via a text message and photo from Caregiver AA. Manager S showed Surveyor the photo and the open area was bright red and open. Manager S said, "I had [Caregiver AA] bandage it, I was out of town." Surveyor asked if the physician was informed of the injury and Manager S said, "I didn't have access to email and I have another job and school work and it was a holiday. I wasn't gonna email them, it was on a holiday and we had a meeting scheduled Monday (07/06/2026) with [the MCO] already. I told [staff] to keep an eye on it for infection." Manager S said staff have been keeping the area clean and bandaged, but neither those instructions nor charting of the care are documented anywhere in Resident 11's record. Surveyor asked what Manager S would have done if s/he had been in work status and s/he replied, "I would have emailed [the MCO] the picture and let them know to see if [s/he] could get home health because [s/he] denies the hospital." Surveyor asked if anyone offered or encouraged Resident 11 to have the wound medically assessed and Managers S said, " I don't know, I wasn't here. I was 2.5 hrs away at my other job...I'm sure [Caregiver AA] did...As you heard today [s/he] hates doctors.." Manager S confirmed there was no documentation of efforts to offer or encourage Resident 11 to be medically assessed and [his/her] physician was not informed of the injury.	{N 431}		

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{N 431}	Continued From page 59 During the exit conference on 07/08/2026 at 1:53 PM, Surveyor shared concerns with Manager S and Co-Owner K about the 4 day delay in taking steps to ensure Resident 11's wound was medically assessed and treated along with concerns the bed frame posed ongoing risk for injury. Manager S shared an email with Surveyor that s/he received from Registered Nurse (RN) BB with the MCO. The message read, "I will call [Resident 11 physician and request a Home Health referral for wound care. As soon as you can, please pad the edge of [his/her] bed, or cover with a mattress and add padding to the side of the mattress by the wall..." Manager S and Co-Owner K expressed skepticism about the intervention and Manager S told Co-Owner K "someone else has to take care of it" because s/he was going to be off until 07/13/2026. Surveyor received and reviewed records from the MCO On 07/10/2026. Resident 11's member centered plan (MCP) identified a history of skin integrity concerns including edema and recent hospitalizations due to infected wounds on his/her feet. On 07/13/2026 at 11:30 AM, Surveyor conducted an interview with Resident 11's MCO care team including Registered Nurse (RN) BB and Care Coach CC. Both stated they first learned of the wound during a routine visit on 07/06/2026. RN BB said s/he assessed the wound and then facilitated the home health referral with Resident 11's physician. RN BB said s/he learned earlier today (7 days after the referral) that home health still had not contacted the facility and Manager S had not reached out to inform the care team of the delay. RN BB and Care Coach CC agreed that given Resident 11's skin integrity concerns	{N 431}		

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{N 431}	Continued From page 60 and past skin infections, prompt assessment and treatment of any wounds is important. RN BB also stated during the check in today, Manager S said s/he had not yet made adjustments to the metal bed frame to protect Resident 11 from additional injuries. Cross Reference: N0196 DHS 83.14(2)(a) Licensee Ensures Facility Complies with Laws	{N 431}			